Medicare Part D | Executive Spending & Utilization Report

National CMS drug spending data, 2020–2024 | November 2026

2024 gross spending	2024 YoY spending	2024 YoY fills
$288.67B	**+4.66%**	**+5.87%**

Executive summary

Reported Medicare Part D gross spending grew from **$275.81 billion in 2023 to $288.67 billion in 2024 (+4.66%)**. Prescription fills increased **5.87%**, while aggregate gross spending per fill fell from about **$170.60 to $168.65 (about -1.14%)**. The utilization increase more than offset the fall in reported average spending per claim. This mathematical relationship does not identify specific causes or demonstrate changes in medication prices.

Business objective and source

Assess national Medicare Part D spending concentration, changes in utilization, and drug-level financial trends using CMS Medicare Part D Spending by Drug (2024 annual extract, with 2020–2024 fields). These reported values are national gross drug expenditures, not an individual health plan's net costs.

Data engineering and validation

The source contains **14,536 rows and 46 columns**. It combines overall drug summaries with manufacturer-specific detail. To avoid duplicate aggregation, program totals use only **3,625 Overall rows**. The source was reshaped to **72,680 drug-year records** for MySQL analysis. The unfiltered 2024 total was $577.34B; the correctly filtered total was **$288,667,842,058.40**. There were no exact duplicate source rows or duplicate Overall brand/generic keys. All 3,625 Overall records reconciled spending per claim within $0.01; the largest difference was about $0.00000412. In 2024, 395 Overall rows carried CMS outlier flags; 18 flags were missing.

Key finding 1 | Concentration

The ten largest 2024 drug spending records contributed **$82.30 billion (28.51%)** of national reported gross expenditures. Eliquis ranked first (~$20.77B), followed by Ozempic (~$12.97B) and Jardiance (~$11.44B). These drugs warrant targeted monitoring, not an automatic recommendation to restrict coverage.

Key finding 2 | Utilization and average claim spending

Atorvastatin calcium had roughly 73.35M fills and $10.31 average spending per fill, while Eliquis had roughly 24.06M fills and $863.42 average spending per fill. High prescription volume and high gross spending are different rankings. Among products with at least 1,000 fills (an analyst-chosen threshold), Amvuttra ranked highest by spending per claim (~$118,225.37) with 1,192 fills.

Key finding 3 | Matched-drug increases

Among drugs with spending reported in both 2023 and 2024, Mounjaro had the largest absolute increase (~$3.98B), followed by Ozempic (~$3.78B) and Jardiance (~$2.60B). Their fills grew 180.29%, 50.36%, and 39.43%, while spending per fill declined 4.26%, 6.18%, and 7.22%, respectively. This is consistent with growing utilization more than offsetting lower average spending per claim. Matched-drug results do not account for every change in the reported national total.

Recommendations

1. Monitor dollar and percentage changes together. **2.** Pair overall expenditures with utilization and cost-per-fill measures. **3.** Continue explicit aggregation, reconciliation, and missingness controls. **4.** Require plan-level net pricing, clinical review, and patient access evidence before any formulary or cost-management decisions.

Interpretation limitations

Gross amounts exclude confidential manufacturer rebates and price concessions. Year-to-year reported drug populations vary due to historical missingness and suppression. Aggregated drug records do not trace patients, determine appropriateness of prescribing, or isolate drug-price changes. Flagged records require contextual review rather than automatic removal.

Sources and interactive output

CMS Medicare Part D Spending by Drug data, Data Dictionary and Methodology (June 2026). Published Tableau Public dashboard: https://public.tableau.com/app/profile/janessa.reed/viz/MedicarePartDSpendingUtilizationAnalysis/MedicarePartDSpendingUtilizationAnalysis